

Public Health Emergency Operations Center
"COUSP DRC"

MVE-17 Incident Management System

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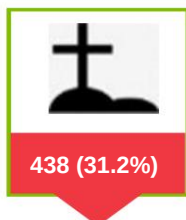
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°047/MVB_30/06/2026

Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (36)	- Ituri (24/36): Aru, Aungba, Bambu, Bunia, Damas, Drodro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima and Lolwa. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. - South Kivu (1/34): Miti-Murhesa
Reporting date	June 30, 2026
Publication date	July 1, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



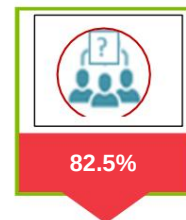
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 2 provinces

* Data updated and subject to change following the cleaning, reclassification and harmonization of the DHIS2 database, including the consolidation of information from health zones and laboratories and structures for handling cases.

Suspicious deaths occurring in the community are subject to investigation and further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

- **Seventy-three (73) new confirmed cases, including 25 deaths**, were reported during the day on June 30, 2026, in the provinces of Ituri (**69 cases, including 22 deaths**) and North Kivu (**4 cases and 3 deaths**). These confirmed cases come from the health zones of **Bunia (29), Nizi (20), Rwampara (6), Mongbwalu (6), Komanda (2), Nyankunde (1), Tchomia (1), Mandima (1), Nia-Nia (1), Kambala (1) and Lolwa (1)** in Ituri and **Katwa (2), Beni (2)** in North Kivu.
- A new health zone has been affected in Ituri, namely the **Lolwa health zone**.
- Investigations conducted following the events of June 29, 2026, and reported on June 30, 2026, led to the notification of two confirmed cases of Ebola virus disease (EVD) in the Wamba Health Zone, Haut-Uélé Province, including one death and one surviving case who fled from the Nia-Nia Health Zone. In addition, Tshopo Province reported a confirmed case of EVD in a 24-year-old woman, six months pregnant, who died at PK 51, the epicenter of the outbreak in the Nia-Nia Health Zone.
The deceased woman's body was clandestinely transported by motorcycle from PK 51 to Kisangani, where a sample taken from her body tested positive. Following investigation, all of these cases were linked to the Nia-Nia Health Zone, Ituri Province, specifically the Abanagomu health area, and are included in the epidemiological situation of this health zone.
- **Fourteen (14) deaths** were recorded among Ebola virus disease (EVD) patients in Ebola treatment centers (ETCs) in the provinces of Ituri and North Kivu. Of these deaths, nine (9) were reported in Ituri, including five (5) at the Bunia ETC, two (2) at the Mongbwalu ETC, one (1) at the Bambu ETC, and one (1) at the Elikya ETC. Five (5) other deaths were reported in North Kivu province, including four (4) at the Katwa ETC and one (1) at the Beni ETC.
- **Nineteen (19) patients** with Ebola virus disease in Ituri province have been declared recovered after receiving negative results in follow-up tests. These recoveries were recorded in the

different care structures, including two (2) at the CME, six (6) at the CTE Mongbwalu, seven (7) at the CTE Elihya and four (4) at the CTE ISTM.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million, with massive internal displacement and cross-border flows to neighboring countries.

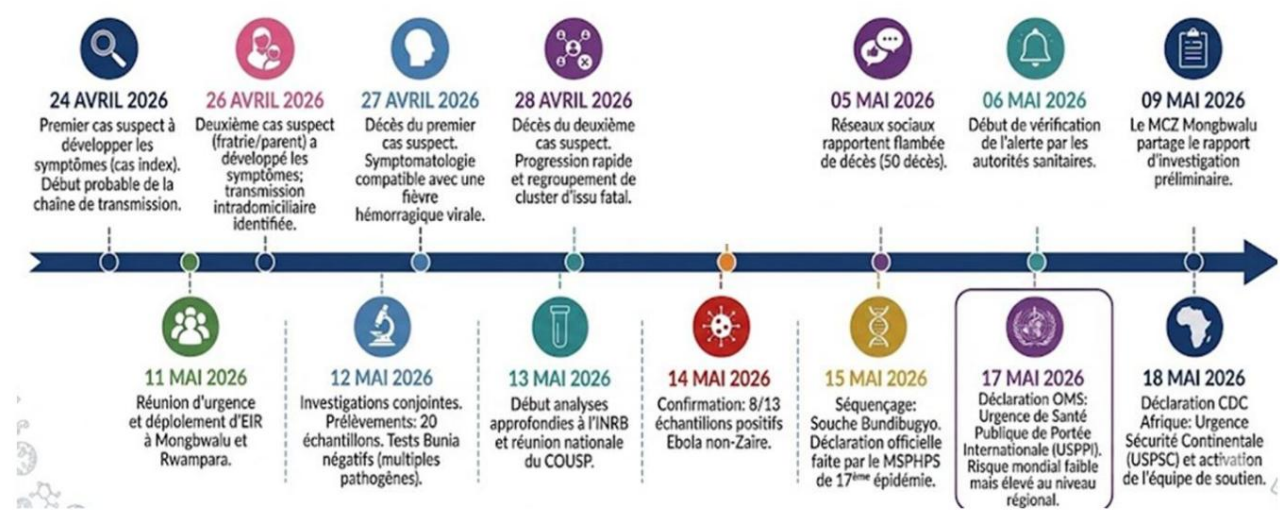


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 30, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of June 30, 2026.

Province	Confirmed cases	Death (confirmed)	Lethality	Affected health zones	New cases confirmed (24h)
Ituri	1283	366*	28.5%	24 out of 36 (66.7%)	69
North Kivu	120	71*	59.2%	11 out of 34 (32.3%)	4
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	1,406	438	31.2%	36 out of 104 (34.6%)	73

* Nine deaths in Ituri and five deaths in North Kivu were recorded among patients with Ebola virus disease (previous confirmed cases) in the CTE/CT.

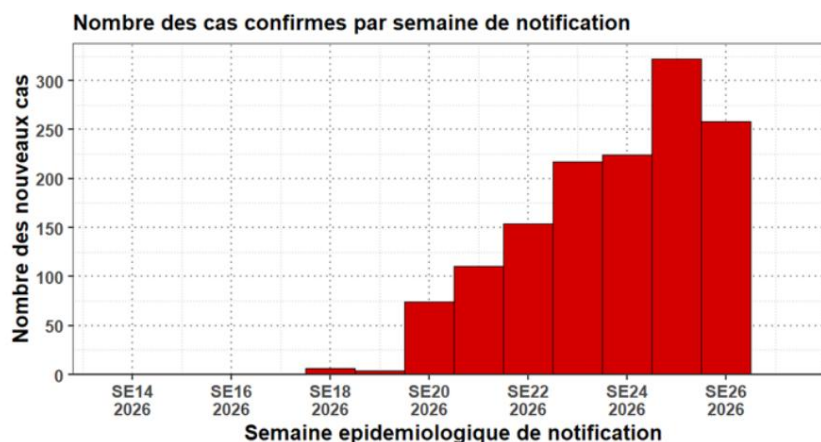
A new Health Zone, specifically the Lolwa Health Zone, was affected on June 30, 2026. The total number of affected Health Zones has thus risen to 36 out of 104 across the three provinces.

affected. The province of South Kivu has not recorded any new confirmed cases since the notification of the last confirmed case on May 26, 2026.

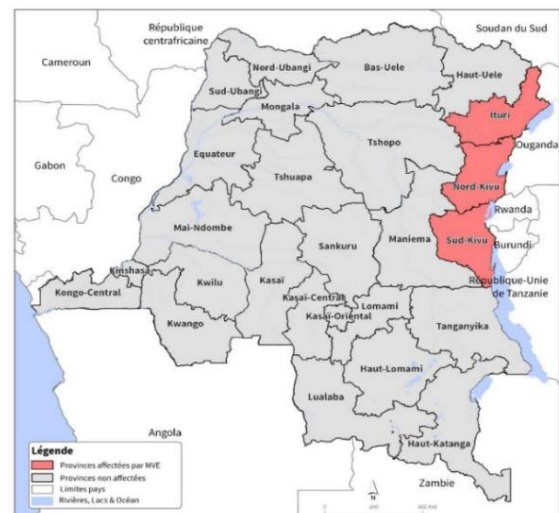
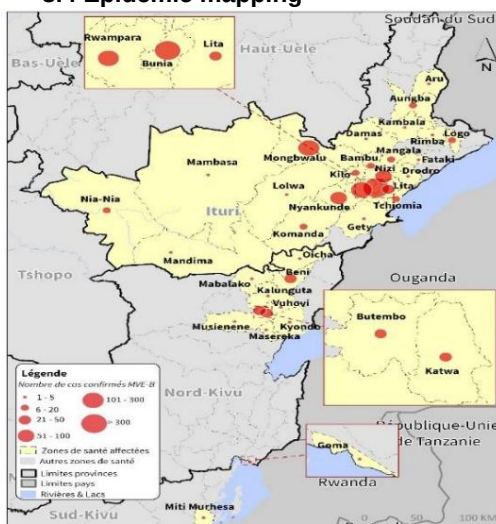
3.2 Distribution by health zone (Cumulative as of 30/06/2026)

- **Ituri Province (1,283 cases):** Bunia (387), Rwampara (301), Mongbwalu (264), Nyankunde (95), Nizi (61), Lita (29), Mangala (24), Bambu (16), Komanda (18), Tchomia (14), Kilo (9), Logo (7), Aungba (6), Nia-Nia (12), Damascus (5), Rimba (3), Aru (3), Drodoro (3), Mambasa (2), Gety (1), Kambala (2), Fataki (1) and Mandima (2), Lolwa (1). Seventeen (17) additional cases are awaiting precise breakdown by health zone in the province.
- **North Kivu Province (120 cases):** Katwa (47), Butembo (34), Beni (24), Oicha (3), Kyondo (3), Kalunguta (2), Musienene (3), Goma (1), Masereka (1), Vuhovi (1) and Mabalako (1).
- **South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Time-bound analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting ongoing community transmission of the disease, although the number of cases appeared to be stable during epidemiological weeks 23 and 24. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capabilities) contributed to early detection, confirming continued and increased community transmission by week 25. Compared to all the weeks in which cases have been recorded since the beginning of the epidemic, week 25 represents the peak so far, with more than 300 cases per week.



3.4 Epidemic mapping



Figures 2 and 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of June 30, 2026

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (June 30, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR, %)
ITURI	1283	366	28.5%
Bunia	387	94	24.3%
Rwampara	301	61	20.3%
Mongbwalu	264	126	47.7%
Nyankunde	95	16	16.8%
Nizi	61	19	31.1%
Lita	29	9	31.0%
Komanda	18	5	27.8%
Bamboo	16	5	31.3%
Kilo			11.1%
Mangala	9 24	1 12	50.0%
Tchomia	14	2	14.3%
Damascus	5	0	0.0%
Aungba	6	2	33.3%
Rimba	3	0	0.0%
Aru	3		33.3%
Mambasa	2		50.0%
Logo	7	1	0.0%
Drodro	3	1	100.0%
Gety	1	0	0.0%
Kambala	2	3	100.0%
Nia-Nia*	12	0	41.7%
Fataki		2	0.0%
Mandima	1 2	5 0 1	50.0%
Lolwa			100.0%
Other areas not yet identified	1 17	10	0.0%
NORTH KIVU	120	71	59.2%
Butembo	34	15	44.1%
Katwa	47	33	70.2%
Blessed	24	15	62.5%
Oicha		2	66.7%
Kalunguta	3 2	1	50.0%
Kyondo	3	2	66.7%
Goma	1	0	0.0%
Masereka	1	0	0.0%
Vuhovi	1		100.0%
Mabalako	1	1	0.0%
Musienene	3	0	66.7%
SOUTH KIVU	3	2 1	33.3%
Miti-Murhesa	3	1	33.3%
TOTAL	1,406	438	31.2%

* Imported cases in the provinces of Tshopo and Haut Uélé are counted in the Niania health zone.

In the past 24 hours, **73 new confirmed cases, including 25 deaths, have been recorded** in the provinces of Ituri (69 cases, including 22 deaths) and North Kivu (4 cases, including 3 deaths), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) epidemic in the Democratic Republic of Congo. Thus, the cumulative total stands at 1,406 confirmed cases and 438 deaths, expressing an overall case fatality rate of 31.2%.

Provincial dynamics and case concentration

The **Ituri** province alone accounts for **91.2%** of cases (n=1,283) and **83.6%** of deaths (n=366), representing a case fatality rate of **28.5%**, making it the absolute epicenter of the epidemic. The health zones of Bunia (387 cases), Rwampara (301 cases) and Mongbwalu (264 cases) remain the most affected, accounting for approximately 74.2% of reported cases at the provincial level and 67.7% at the national level. Within these same health zones, moderate case fatality rates were observed in Rwampara (20.3%) and Bunia (24.3%), while Mongbwalu **also** experienced a high case fatality rate of **47.7%** (126 deaths out of 264 cases), suggesting persistent challenges in early intervention and access to care. It should be noted that 17 additional cases (with no associated deaths) have been reported, while the health zones of origin remain to be identified, which could indicate a geographical expansion requiring further investigation.

Severity of the epidemic in North Kivu

A total of 120 confirmed cases and 71 deaths have been reported in North Kivu province since the start of the outbreak, representing a **case fatality rate of 59.2%**. This province presents the most concerning profile in terms of severity, with a case fatality rate significantly higher than the national average. This high case fatality rate is concentrated in the health zones of **Katwa** (70.2%), **Beni** (62.5%), and **Oicha** (66.7%). The Katwa health zone has the highest number of reported cases (n=47), followed by the Butembo health zone (n=34) with a case fatality rate of 44.1%. A case fatality rate of 100% was exceptionally observed in the Vuhovi health zone (which has a small population).

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Continued supervision of field response teams.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of June 30, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Report (D-1)		20	0	2	22
New Alerts		387	814	11	1,212
Total alerts for today		407	814	13	1,234
Alerts investigated		212	814	9	1035
Investigation rate		52.1%	100.0%	69.2%	83.9%
Alerts confirmed	Alive	167	43	1	211
	Deceased	56	34	0	90
Total suspected cases for today		223	77	1	301

- By the end of the day on June 30, 2026, **1,234 alerts** had been reported across the three provinces, **1,035 (83.9%)** were investigated, **301 suspected cases including 90 deaths were identified** (Table 3).

Table 4. Contact tracing of confirmed cases as of June 30, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri*	9,238	7,631	82.6%
North Kivu*	2,408	1974	81.9%
Total	11,646	9,605	82.5%

* The follow-up rate was calculated only for health zones that submitted reports (completeness: 90.6%). The health zones of Gety and Mandima (Ituri) and Musienene (North Kivu) did not have data available at the time of report consolidation.

A multidisciplinary team has been deployed to the two provinces that have reported confirmed cases (Tshopo and Haut - Uele) to conduct investigations and public health interventions around the reported cases. Some contacts have been transferred to the NiaNia Health Zone to ensure continuity of their follow-up.

- Continued in-depth investigations and exhaustive listing and systematic follow-up of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and followed per case).
- **389 new contacts** were listed, including **353** in 8 health zones in Ituri province and **36** in 2 health zones in North Kivu province.
- **104 contacts successfully completed their 21-day follow-up**, including **83** in 1 health zone in Ituri province and **31** in 1 ZS of the province of North Kivu.

Update on checkpoint and point of entry (PoC/PoE) activities

- An alert was raised at PoC Pluto (ZS Mongbwalu) on 26/06/2026, and the analysis result came back positive.
- **Ituri** : Two (2) alerts were received, one of which was a living person. The living person was confirmed and referred to Bunia General Hospital, and the deceased was intercepted at the Odumoni Point of Control, released after document verification, and taken to the morgue.
- **In North Kivu**: Four (4) alerts were detected and notified, including 1 living person at the Mavivi Barrier PoC and 3 lifeless bodies intercepted at the Mukulia, PK5 and Kasitu PoCs. 3 alerts (deaths) were validated and referred to the morgue, representing a validation rate of 75%;

Table 5. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of June 30, 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	98.1% (54/55)	100.0% (14/14)	98.5% (68/69)
Number of people who went through PoE/PoC	166 171	63,837	230,008
testing, % of travelers	98.2%	97.5%	97.9%
screened, % of travelers who washed their	97.9%	97.5%	97.7%
hands, % of travelers made aware of the issue	99%	97.5%	98.2%
Number of alerts notified	2	4	6
Number of validated live alerts	1	0	1
Number of lifeless bodies intercepted today	1	3	4

Number of problematic contacts intercepted today	0	0	0
Percentage of validated live alerts referred to the CT/CTE	100.0%	0.0%	0.0%
% of lifeless bodies swabbed	100.0%	100.0%	100.0%
Number of positive results of referred suspected cases	1	0	1

4.3. Laboratory

- **Ituri** : 109 samples collected and analyzed (**positivity 63.3%; n=69**).
- **North Kivu** : 135 samples collected and analyzed, (**positivity 3%; n=4**).
- **South Kivu** : 1 sample has been collected and is currently being analyzed.

4.4. Infection Prevention and Control (IPC)

Ituri: 29/27 households were identified and decontaminated around confirmed cases; 3 ESS identified and decontaminated around confirmed cases in the health zones of Bunia, Rwampara, Mongbwalu, Komanda; 9 decontaminations of public and specific places: Sokimo office, DGPI, COUSP; 26/29 EDS carried out with 2 postponements; PCI/WASH evaluation of 9 ESS in the health zones of Mongbwalu, Bunia, Lita, Nyankunde, Rwampara: PS Katoto: PS Ushindi 38.7%, PS Maranatha 45.2%, CS Komanda 76%, PS sante d'abord 34%, Dispensaire, Afia Bora 30.6%, CH Unibu 71 TM green World Hospi 58.1%, PS Bora Uzima 8.1%, PS Uhuru 32.2%, PSA Tout est grace 34%, PS Yesu ni Jibu 23%, PS Siloe 34.9%, CS hoho 64.2%, CSR Lembabo 72.5%, CM le Sauveur 17.7%; Decontamination of 55 means of transport (ambulance, hearses) and briefing of 33 providers on Ebola Virus Disease (EVD).

North Kivu : WHO provides 5 vehicles to teams in the 4 health zones: Katwa, Butembo, Musienene and Kyondo, for IPC activities.

4.5 Holistic care

- **Fourteen (14)** deaths were recorded among patients with Ebola virus disease (EVD) in Ebola treatment centers (ETCs) in the provinces of Ituri and North Kivu. Of these deaths, nine (9) were reported in Ituri, including five (5) at the Bunia ETC, two (2) at the Mongbwalu ETC, one (1) at the Bambu ETC, and one (1) at the Elikya ETC. Two (5) other deaths were reported in North Kivu, specifically at the Katwa ETC (4) and the Beni ETC (1).
- By the end of the day on June 30, 2026, 738 hot meals had been served, including 141 to cases confirmed, 164 to suspected cases, 383 to PPLs and 50 to accompanying persons, Twenty (1) dry rations were also distributed.
- **North Kivu**
Training of 51 CTE Beni providers on patient care with IMC support and briefing of supervisors on PCI with IMA support.

Mental health activities and psychosocial support

Table 6. Key indicators of mental health and psychosocial support activities as of June 30, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	2 (66.7%)	1 (100.0%)	0
Confirmed cases currently being monitored that have received interventions SMSPS	79 (61.7%)	9 (100.0%)	0
New suspected cases that benefited from SMSPS interventions	11 (55.0%)	5 (100.0%)	1 (100.0%)
Suspected cases under follow-up that have received interventions SMSPS	88 (63.3%)	12 (100.0%)	1 (100.0%)
Laboratory results have been announced.	14	23	1
Including positive ones	0	1	0
Number of children separated in daycare who benefited from interventions SMSPS	0	0	0
Number of children separated in the community who benefited from SMSPS interventions	0	3	0
Number of accompanying persons at the CTE who benefited from interventions SMSPS	56	2	0
Number of PPLs who benefited from SMSPS interventions	11	2	0
Number of household and community members who benefited from SMSPS interventions	31	1	0
Number of EDS supported	16	3	0

4.6 Continuity of care

Provision of essential medicines for PMA and PCA in other health zones.

Psycho-social support for affected PPLs and their dependents.

Support for non-cases (children, mothers and other vulnerable individuals) within the continuity of services actions

4.7 Risk communication and community engagement

Ituri: Covered in stickers and posters with educational messages on Ebola prevention, 8000 motorcycles and 150 taxi cars crisscrossed the main thoroughfares of the city of Bunia.

Six (6) social mobilization activities (community dialogues and mass awareness) reaching 3904 (1076 men and 2828 women) on preventive measures for Ebola virus disease, community-based surveillance, care, decontamination and EDS in the health zones of Mongbwalu, Nyankunde, Nizi, Rwampara, Nia-Nia, Aungba, Bambu, Aru; 2 educational talks and home visits organised by Recos, 525 households were reached and 4,275 people reached (1771 men and 2504 women) in the health zones of Bunia, Nyankunde; Briefing of 42 providers on community-based surveillance, CREC with practical exercises on the organisation of home visits, facilitation of educational talks in their respective environments and the use of tools (community definition of cases, contact registration form, contact tracing form, and home visit form); 19 broadcasts of spots and messages on preventive measures for Ebola Virus Disease.

Community feedback: The establishment of community liaisons from the site to strengthen monitoring and awareness; the construction of latrines and the installation of handwashing facilities,

due to their near absence; improved access to clean water; Ebola business for doctors and nurses to make money; caregivers injecting other products to cause Ebola

Support to the pillars of the

response : Surveillance: Support to the pillars of the response: Surveillance: Contact tracing and investigation around 7 cases and 51 people around a case of death at the TSERE site.

PCI: Support to EDS and PCI teams for the cases of deaths that occurred at the Juhudi CTE in the Nia-Nia Health Zone.

North Kivu: 1770 out of 6750 registered people were involved in community actions; community dialogue with 15 community leaders on strengthening Ebola prevention strategies in the Irango health district in the Vuhovi health zone; Four mass communication sessions for 398 Christians, including 243 from the UKWELI NA BWANA church (Wanamahika health district), 53 members of the MESS church (RUGHENDA health district), 78 Christians from the CHRISCO church and 24 members of the ECC, on the risks of contamination and prevention measures against Ebola in the Katwa health zone; Educational talk with 13 people from Nzuma/Beni health zone to raise awareness among youth leaders in order to reduce community incidents and resistance that led the neighborhood chief and the IT to flee their homes; 6901 households were visited by community actors (CAC members, RECO, Leaders, Volunteers, UReporters, etc.) out of the 25342 planned.

4.8. Logistics

Ituri

- Receipt of laboratory reagents, medications, communication supplies, and computer equipment (60 laptops and 8 printers); delivery of medications and infection prevention and control (IPC) supplies to the Mangala Health Zone, the CTE CME/Bunia, and the Bunia Provincial Laboratory; provision of 26 vehicles and 4 ambulances to the various pillars, and fuel supply for four ambulances and two jeeps of the security pillar, totaling 360 liters. Development work continues at Bunia General Hospital, Rwampara General Hospital, Mongbwalu General Hospital, and at the Nizi General Hospital tent.

North Kivu

- Delivery of Swab tubes to the Oicha Health Zone
- Participate in driver briefings on barrier measures and in EPP briefings on PCI
- Participation in the Ebola Response Coordination meeting
- Deployment of PCI teams in the field to monitor the practical application of the theory learned in person

4.9. Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : continued integration of PSEA messages into briefings for field actors, with an emphasis on teams in direct contact with patients, families, PPLs and affected communities.



Data and IT management training on DHIS2 Tracker in the Nyankunde zone with support from Bluesqaire

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity of the CTEs and saturation in North Kivu (138.8%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTE, deploy additional temporary structures
3. Performance of the monitoring of contacts not optimal (81.3% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision

4.	Low reporting of alerts in certain affected zones	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5	confirmed cases not yet broken down by health zone (17 case)	Major gaps in the reconstruction of transmission chains	Conducting a data reconciliation, Remove any potential duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.). Active verification in the field.
6	Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (96 cases) HCW (including 19 deaths, case fatality rate 19.7%)	Urgent supply, strengthening of IPC training
8	Stockpile of unanalyzed samples (backlog) at the laboratory in North Kivu and delayed diagnosis	Delayed confirmation, high lethality (54.2%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities
9	Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10	Insufficient and weak coordination use of COUSP frames	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, make operational the Provincial COUSP
11.	Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12	Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13	Continuity of essential services (CEHS) compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14.	High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

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**Avec l'appui technique de l'Organisation Mondiale
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